

DETAILED PRACTICAL GUIDE

Caring for women subjected to violence: a WHO curriculum for training health-care providers

WHO · English · 2019

Important: This is a detailed unofficial reading guide, not an official translation. Consult the original publication for complete requirements and context.

Based on the original publication

<https://iris.who.int/bitstream/handle/10665/330084/9789241517102-eng.pdf>

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01 Purpose and scope

- A training guide for health-care providers on recognising violence, offering first-line support, providing clinical care and connecting women with safe services.

02 Key guidance

- Create private consultation conditions and never ask about violence when a partner, relative or interpreter chosen by the partner is present.
- Use first-line support: listen closely, inquire about needs, validate, enhance safety and connect to support.
- Ask only when clinically relevant and when a safe response pathway exists; routine questioning without capacity to respond can cause harm.
- Assess immediate danger, threats, weapons, stalking, strangulation, escalating violence and threats involving children.
- Treat injuries and health consequences without making police reporting a condition of care.
- Document objectively, securely and with consent; distinguish observations from allegations and avoid judgmental language.
- Explain confidentiality and mandatory-reporting limits before disclosure, including special rules affecting children or people lacking capacity.
- Practise through supervised role-play, reflection and referral exercises; provider attitudes and bias directly affect quality of care.

03 How to use this guide

- This guide turns the source publication into a practical reading aid. It is designed for survivors, trusted supporters, health and social-service staff, and programme managers. It does not replace emergency services, medical care, legal advice, or the complete source publication.
- Use only the parts that fit the person's situation. Preserve her choice, privacy and pace. If a suggested action could expose browsing, calls, documents or location to an abusive person, first choose a safer device, place or time.

04 Survivor-centred principles

- Safety comes first. Ask what would make the next hours or days safer and avoid steps that may provoke retaliation. Never require a disclosure in order to offer basic help.
- Respect autonomy and informed consent. Explain realistic options, benefits, limits and possible risks; the survivor decides what happens next unless an immediate legal safeguarding duty applies.
- Protect confidentiality. Collect the minimum information, restrict access, agree on safe contact methods and explain any limits before information is shared.
- Provide equitable access. Consider disability, language, age, migration status, poverty, rural isolation, sexual orientation and other barriers without stereotyping.

05 Practical response sequence

- Begin with immediate safety and urgent health needs. Ask privately, without the suspected perpetrator present, whether it is safe to talk and whether the person needs emergency care.
- Listen without interrogation. Validate what happened, avoid blame, and do not demand a chronological account or proof. Record only what is necessary and use the survivor's own words.
- Offer a small number of verified options: medical care, safe accommodation, psychosocial support, legal information, police or protection services where appropriate. Explain that accepting one service does not require accepting all others.
- Make a warm referral when consent is given: confirm the service is open, accessible and safe; explain what to expect; and avoid sending the survivor repeatedly between agencies.
- Agree on follow-up and a safe way to communicate. Do not leave identifying messages, send revealing notifications or store files under obvious names if devices may be monitored.

06 Safety and quality checks

- Do not confront the suspected perpetrator, mediate a violent relationship, promise an outcome you cannot control, or pressure a survivor to leave immediately.
- Do not use unverified phone numbers or referrals. Services may close, change eligibility rules or share information in unsafe ways; confirm details before publication or referral.
- Organisations should train staff, supervise difficult cases, create confidential documentation rules, maintain updated referral maps and offer staff support to reduce burnout and harmful practice.

Action checklist

- Is it safe to speak, read or keep this document right now?
- Have urgent injuries, pregnancy concerns, sexual-violence care and suicide risk been considered?
- Has the survivor chosen what information may be recorded or shared?
- Are interpretation, disability access, childcare, transport and cost barriers addressed?
- Are referrals verified and is there a safe follow-up plan?
- Has the person been told that the violence is not her fault and that she can decline any option?